

Nosebleed (Epistaxis)

History

- When did nosebleed start?
- How severe is bleeding?
- Did patient have any nasal trauma or foreign bodies?
- Has patient had recent signs / symptoms of sinusitis?
- Has patient been using topical nasal medications / steroids?
- Does patient have a bleeding disorder or take blood thinners? Plavix? Pradaxa? Xarelto? Eliquis? Warfarin? Aspirin?
- Does patient have hypertension? Has blood pressure been high recently?

Physical

- Bleeding Severe (arterial) or mild (oozing)?
- Vital signs – Is patient very hypertensive? Tachycardic?
- Is patient anxious?
- Does patient have airway compromise from massive bleed?
- Other signs of bleeding disorder? (Bruising? Bleeding gums?)

Note:

- Most (90%) of nosebleeds are from the anterior nose and are usually self-limited and rarely life-threatening. However, there are some nosebleeds that are from the posterior nose and can be life-threatening and require emergency treatment.

EMT/AEMT

- Follow General Medical Care Guideline

Active Bleeding

- Provide immediate direct pressure – by pinching anterior portion of nose with moderate force constantly for 15 minutes

EMT/AEMT - Expanded Scope

- **Afrin** 2 sprays in each nostril - then hold pressure
- **NS/LR** IO per Shock Guidelines
- **TXA** 500mg IN with atomizer or Nebulized or soaked gauze packing

Paramedic

- **Afrin** 2 sprays in each nostril - then hold pressure
- **NS/LR** IV/IO per General Medical Care Guideline
- **TXA** 500mg IN with atomizer or nebulizer or soaked gauze packing
 - May use 2g IV if HR >110 or Systolic BP <90 mmHg

Active Bleeding due to trauma

- Consider Pain Management Guideline